



WNHS PROCEDURE	
Electroconvulsive Therapy in the Postnatal Period	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Mother Baby Unit (MBU)

Contents

1. Aim	1
2. Risk	1
3. Key Points	2
4. Process	2
Decision making responsibility	2
Referral	3
Consent.....	3
Approvals	3
Considerations for treatment.....	3
5. Post Treatment.....	5

1. Aim

The purpose of this guideline to ensure staff follow the correct process in the preparation and care of all MBU patients prescribed Electroconvulsive Therapy (ECT) in accordance with the [Mental Health Act \(MHA\) 2014](#) and comply with the [Chief Psychiatrist's Practice Standards for the Administration of Electroconvulsive Therapy 2015](#).

This guideline is to be read in conjunction with "[The ECT Guide: the Chief Psychiatrist's guidelines for the use of ECT in WA](#) 2006.

Please note: the use of the word 'Prescriber' throughout the document refers to the treating consultant psychiatrist from Mother and Baby Unit, King Edward Memorial Hospital. The 'Interventionist' refers to the specialist consultant psychiatrist at SCGH Neuromodulation Service performing the ECT.

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☐
Breach National/State/Mandatory Policy	☐	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☐
Other	☐		

3. Key Points

- Patients and their partner/support person must be fully informed of their rights and responsibilities with agreement to treatment sought and consent received prior to ECT (see patient information sheet "[Receiving treatment for a mental illness](#)").
- Indications for ECT include but are not limited to:
 - Severe depression
 - Severe mixed affective disorder
 - Severe mania or catatonia
 - Postpartum psychosis
 further considerations may include:
 - The physical health of the mother is at serious risk, or may impact on the baby
 - Insufficient time to wait for improvement with psychotropic medication
 - Patient and partner's preference
- If a woman is breastfeeding then special consideration should be made of the risks and the recommended practice guidelines for ECT ([RANZCP Professional Practice Guidelines](#)).
- If other comorbidities such as recent myocardial infarction, head injury, chest infection, complicated chronic obstructive airways disease or cerebral vascular accident, the opinion of a specialist physician should be sought.

4. Process

Decision making responsibility

- The MBU Prescriber takes clinical responsibility for the decision to treat a patient with ECT. The decision will be made in conjunction with the MBU treating team responsible for the care of the patient, the SCGH specialist anaesthetist and specialist psychiatrist.
- If breastfeeding then advice should be sought on the risks and recommendations for exposure to the specific anaesthetic agents through breastmilk. It may be recommended that a woman expresses and discards her milk for a period after the anaesthetic.

Referral

- MBU patients requiring ECT will be referred to SCGH Isdell Day Procedure Unit (DPU) by contacting the Electroconvulsive Therapy Nurse Coordinator via email ECTCoordinatorSCGHMHUDDPU@health.wa.gov.au or phone (08) 6383 1114.
 - Isdell DPU Operational Hours: 7:00 – 3:30 Monday, Wednesday, Friday.
 - Tuesday, Thursday contactable via switch on 6383 1000

An ECT information pack will be forwarded to the referrer/MBU staff. The pack contains referral details (ECT Service and Anaesthetics), processes, protocols, procedures, checklists, outcome measures and appropriate Medical Record forms (referral, consent) as well as patient information brochure and patient treatment diary. The packs are also available in the ECT folder in MBU.

Consent

- Patient consent must be gained consistent with the requirements of the:
 - [Mental Health Act 2014](#)
 - [Guardianship and Administrative Act 1990](#)
 - [Chief Psychiatrist's Practice Standards for the Administration of Electroconvulsive Therapy 2015](#) – Standard 4 (page 10)
 - [WA Health Consent to Treatment](#) Policy
 - [SCGH Isdell DPU: ECT Policy](#) and using consent forms from the ECT information pack

Approvals

- Approvals must be sought in accordance with the [Mental Health Act 2014](#)
- If emergency ECT treatment is required for an involuntary or MIA patient in order to save the patient's life or if there is imminent risk likely to result in serious injury, approval must be sought from the Chief Psychiatrist or delegate.
- If ECT is to be administered in pregnancy (> 22 weeks gestation) approval must be sought from the Chief Psychiatrist or delegate.

Considerations for treatment

- Breastfeeding - If the patient is well enough and prior to treatment, consideration will be given to ensuring expressed breast milk is available for the baby whilst the patient undergoes treatment and the period post anaesthetic. Post ECT once the patient is fully recovered (awake, ambulant and no headache) breastfeeding can be resumed, in keeping with post-operative/anaesthetic guidelines.

Nursing process for preparing a patient for ECT

DAY PRIOR TO ECT

Nurse allocated to the care of the patient to check all ECT documentation are completed and available including:

ECT consent form (PMR 73)

ECT treatment form (PMR 73)

ECG tracing

CXR

Bloods (available OL via ISOFT)

- Old notes and any past ECT treatment charts and outcomes.
- Outcome measure completed
- MHA forms (if applicable)
- MHT forms (if applicable)
- OCP approval email if emergency treatment (if applicable)
- SAT guardianship order (if applicable)
- Interpreter booked for first anaesthetic assessment (if applicable)

If the documentation isn't available the day before treatment discuss with treating team psychiatry/RMO.

Benzodiazepines are usually withheld/ceased before ECT, discuss with treating team psychiatry/RMO.

NIGHT STAFF PREPARATION OF PATIENT

Please keep all ECT documentation together and filed behind the ECT tab.

- Fast from midnight
- Wake patient at 0630hrs, give any prescribed pre-ECT medications with sip of water.
- Complete [PMR 74 ECT Treatment Nursing Checklist](#)
- Document baseline observations on the current AORC chart which remains in the patient's file.
- All patient files inclusive of Medication Chart (file) and Diabetics Sheet to accompany patient for treatment.
- Check: Consent Form; ECT treatment form (PMR 73); ECG tracing and AORC chart are available and in the patient's file.
- Jewellery to be removed or taped
- Hair to be clean and dry.
- Dentures/contact lenses can be removed in Isdell DPU treatment room.
- No moisturisers face creams or makeup.

MORNING OF ECT

Patient to be prepared by 0730hrs ready to be transported by allocated nurse to SCGH MHU UU block (isdell DPU).

Note: Medications related to management of blood pressure, reflux, asthma or cardiac stability must be given at least an hour prior to each ECT treatment. Other medications not related to ECT but supporting the wellbeing of the patient may be given at least an hour prior to treatment. This may include antipsychotics for anxiety, or analgesics, please discuss with the team prior.

Allocated nurse to handover patient to ECT staff following ISoBAR protocols and is to remain with patient throughout treatment.

POST ECT

All patients will be recovered in SCGH Isdell DPU by recovery staff until discharge criteria is met.

- The escort nurse will receive patient files and a verbal handover from recovery staff following ISoBAR protocols.
- Recovery will be approximately 2 hours post treatment.
- Patient will be provided a light breakfast as tolerated.
- Once reviewed by medical staff and discharge criteria is met the patient will then be transferred back to hospital/ward of origin.
- Escort nurse will be given copy of the most recent integrated progress notes which documents ECT treatment and recover.
- When the patient is fully alert and orientated encourage routine activities including attendance at the ward occupational therapy program.
- Documentation to reflect rationale for ECT therapy e.g., notable changes in MSE.

Encourage patient to use and bring “Your guide and diary for ECT at SCGH Isdell DPU” where any questions/queries can be discussed (also has useful phone numbers).

This is given to all patients before the commencement of treatment, if patient is unable to write themselves, it can be completed with/by the allocated nurse.

Due to length of stay, it is advisable for the escort nurse to bring their morning tea. Tea and coffee is available at the unit.

5. Post Treatment

- Prior to leaving SCGH the patient will be deemed fit for discharge back to MBU and no additional monitoring will therefore be required.
- At the conclusion of treatment, the escorting nurse will contact the MBU to arrange transport back to the unit (either by ambulance or taxi) and ensure all treatment documentation is completed in the patient’s integrated progress notes.

References

[Mental Health Act 2014](#)

[Chief Psychiatrist's Practice Standards for the Administration of ECT](#)

[Clinician's Practice Guide to the Mental Health Act 2014](#)

[Royal Australian and New Zealand College of Psychiatrists professional practice guidelines for the administration of electroconvulsive therapy](#)

Useful resources (including related forms) (if required)

[PMR 74 ECT Treatment Nursing Checklist](#)

Electroconvulsive Therapy in the Postnatal Period

Keywords	Electroconvulsive therapy, ECT, Mental Health Act 2014				
Policy Sponsor	Head of Department, Mother Baby Unit/ Director Of Psychiatry				
Policy Author	Coordinator of Nursing - Mental Health				
Date First Issued:	01/05/2007	Last Reviewed:	23/09/2021	Review Date:	23/09/2024
Approved by:	MBU Team Business Meeting			Date:	23/09/2021
Endorsed by:	MH Sub-Committee			Date:	11/10/2021
NSQHS Standards Applicable:	☐  Std 1: Clinical Governance ☐  Std 5: Comprehensive Care		☐  Std 6: Communicating for Safety ☐  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Printed or personally saved electronic copies of this document are considered uncontrolled					

The health impact upon Aboriginal people has been considered, and where relevant incorporated and appropriately addressed in the development of this policy (insert ISD Number). (Please refer to the Aboriginal Health Impact Statement and Declaration for Department of Health and Health Service Provider Guidelines – please delete once you have completed this).

This document can be made available in alternative formats on request for a person with a disability.

© Women and Newborn Health Service 2020

Copyright of this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism or review, as permitted under the provisions of the Copyright Act 1968, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.